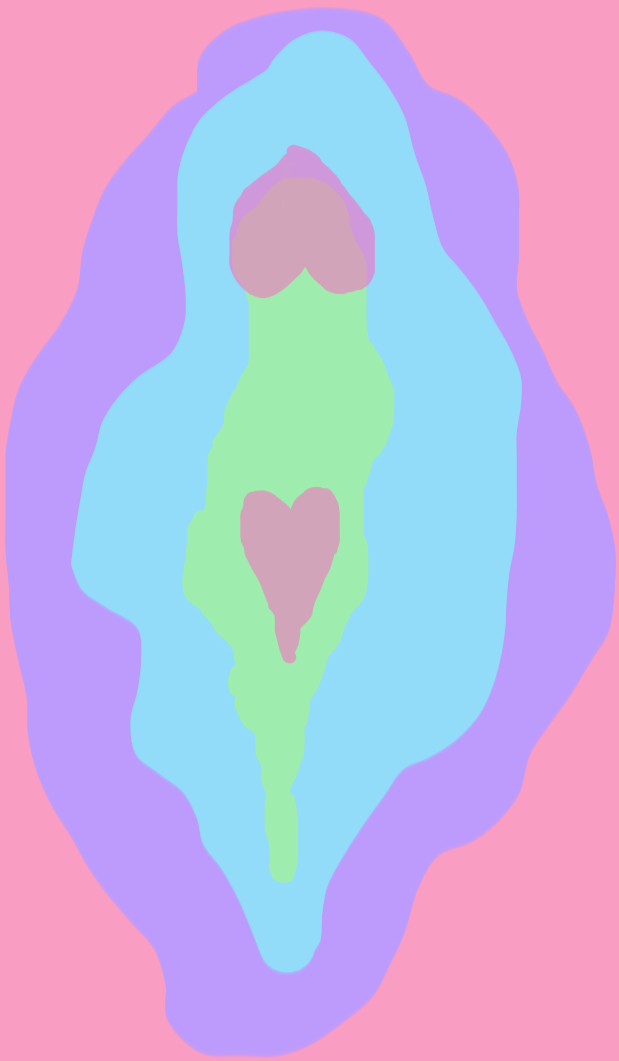
Safe gynecological exam for all young people!

a guide to the (first) gynecological exam



Žena znači Revolucija!



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a guide to the (first) gynecological exam

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for Žena znači Revolucija!
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Introduction

This guide was created to provide all young people key information about a safe (first) gynecological exam.

The guide is divided into three sections:

- before the exam
- during the exam
- after the exam

from gynecological, psychological and legal perspectives (the legal section is referring exclusively to the laws in Serbia, laws in other countries may differ).

If you have questions that are not covered in this guide, you can contact us via:

- zenaznacirevolucija@gmail.com
- Instagram: @zena.znaci.revolucija

This guide is intended for all young people, regardless of gender identity or previous experience with gynecological exam.

Before the exam

What is gynecology and who is a gynecologist?

Gynecology is a branch of medicine that deals with the health of the reproductive organs of women and other people with gynecological health needs.

Gynecology includes:

- Preventive care (regular checkups, Pap test, ultrasound, etc.)
- Diagnosis and treatment of any changes in the reproductive organs of women and other people with gynecological health needs
- Obstetrics (monitoring pregnancy, childbirth, and the postpartum period)

A gynecologist is a person who takes care of the health of the reproductive organs and helps with the prevention, diagnosis, and treatment of various conditions.

Why should we attend a gynecological exam?

For health checkups, prevention, early diagnosis, information about the body and sexual health, and getting support in case of any symptoms.

When should you attend your first exam?

It is recommended between the ages of 15 and 18, or earlier if:

- if your menstrual cycle is irregular or very painful
- if you have symptoms (itching, unpleasant odor, pain, increased discharge, or any changes in the skin or mucous membranes)
- if you need advice on contraception or sexual health

How often should you attend a gynecological exam?

It is recommended **once a year**, even when there are no symptoms, because many changes in our body or health do not show clear signs in the early stages.

Some changes often **do not cause symptoms**, which is why it is important to have a Pap test even when there are no symptoms.

How to prepare for the exam?

You do not need to do anything special. It may help to:

- wear clean, comfortable cotton underwear
- bring a sanitary pad
- avoid vaginal douching, creams, and tampons 24 hours before the exam
- shaving is **not** necessary (the presence or absence of hair is not an obstacle to the exam)

What should the first gynecological exam look like?

The first exam consists of a **conversation** and a **basic examination**.

No procedure is mandatory if there is no medical need for it and your consent.

The gynecologist should talk to you about:

- menstruation (duration, regularity, pain)
- any symptoms you may be experiencing
- your general health condition
- questions about sexual health and contraception
- family medical history (information about diseases and conditions recorded among close family members)

External examination and bimanual examination (if necessary):

An external examination involves examining the skin and mucous membranes without the use of instruments.

A bimanual examination is performed to assess the condition of the uterus and ovaries.

The gynecologist gently presses on the abdomen with one hand while examining through the vagina with the other hand. It may feel uncomfortable, but it is brief and should not be painful.

The conversation is a mandatory part of the examination and you have the right to ask anything you would like to know.

IMPORTANT TO KNOW

In gynecological healthcare, what matters is what happens in sexual relationships, not the gender identity and/or sex of your partner.

Speculum examination (only with your consent):

A speculum is an instrument that allows the examination of the vagina and cervix as well as the collection of swab samples.



The speculum is used to:

- see the cervix
- collect swab samples (e.g. Pap test, vaginal swab)

This part of the examination **is not** mandatory:

- for people who have not had sexual intercourse
- if there is no medical need

You always have the right to refuse this examination or to request a smaller speculum.

Ultrasound examination (if necessary):

An ultrasound is not always performed and it can be:

- **through the abdomen (abdominal ultrasound)** most commonly in people who have not had sexual intercourse
- **vaginal (transvaginal ultrasound)** for people who have had sexual intercourse, if there is a medical need

Ultrasound is performed only if it is needed for diagnosis and with your consent.

In cases of extreme urgency, if there is no time to fill the bladder, an ultrasound can also be performed using a **vaginal probe** through the rectum (large intestine).

IMPORTANT TO KNOW

1. **No procedure should be performed without your consent.**
2. **The examination should be explained to you in advance.**
3. **The examination should proceed at a pace that is comfortable for you.**

How does the examination differ in people who have or have not had sexual intercourse?

- If a person **has not** had penetrative sexual intercourse, an abdominal ultrasound or an external examination is performed.
- If they have, a standard examination and Pap test can be performed, if necessary and if the person feels comfortable.

We asked a gynecologist

What should every young person know before their first gynecological exam?

“Every young person should have age-appropriate sexual education in schools, starting from an early age. Through this education, a person would already know who a gynecologist is, why and when they are needed, what a Pap test, HPV infection and vaccine are, what a gynecological exam looks like, what it includes, and whether the examination differs for someone who has not had sexual intercourse and someone who has.”

Must the examination be uncomfortable or painful?

“Gynecological examinations do not have to be uncomfortable, and they are not painful if the person is relaxed and if there are no local conditions or inflammation. Discomfort and pain during a gynecological examination are the result of fear of the exam, fear of the unknown, lack of preparation, and a rough approach by the gynecologist.”



Prof. dr Aleksandra Kapamadžija

During the exam

What does a gynecological office look like?

A gynecological office is a medical space where you should feel **safe** and **respected**.

It usually contains:

- a gynecological examination chair
- a privacy screen or curtain for changing clothes
- medical instruments (which should be clean and prepared in front of you)
- a gynecologist and possibly a nurse or medical technician

Does the exam have to be uncomfortable or painful?

It does not have to be. The exam **should not be painful**. Mild discomfort is possible, but if you experience significant pain or discomfort, do not ignore it or downplay your experience.

Discomfort most commonly occurs due to:

- fear and tension or a lack of explanation
- an existing gynecological condition
- a rough or insensitive approach

A professional gynecological exam includes:

- a slow and gentle approach
- ongoing communication
- checking in on how you are feeling throughout the examination

A good gynecological exam will not leave you feeling confused, afraid, or ashamed. After the exam, you should know what was done, why it was done, and what the next steps are.

What is unprofessional behavior of a gynecologist?

Unprofessional behavior includes:

- rudeness
- judgmental attitudes
- inappropriate or insensitive comments
- ignoring your questions or pain
- rushing without explanation
- unfriendliness or mocking behavior

If any of the above occurs, you always have the right to change your gynecologist.

Can you bring someone with you to the exam?

You can go to the examination alone or with a trusted person (parent, guardian, friend, partner, etc.).

The right to have a companion is **not** explicitly stated in the law, but it can be interpreted from the Law on Patients' Rights that you have the right to:

- decide who is present during the examination
- refuse the presence of any person you do not want

In practice:

- rules depend on the healthcare facility
- your preference is the starting point
- no one has the right to force you to have a companion or to attend the exam without one, unless there is a justified reason

If you want another person to receive information about your health, you must explicitly give your consent.

What are your rights during the exam?

Before and during the exam, you have the right to:

- a discussion about your health and the reason for your visit
- an explanation of every procedure
- an explanation of the diagnosis
- information about treatment options
- time to think
- the question: **“Do you agree with this?”**

You also have the right to:

- ask questions
- say **no**
- stop the examination at any time
- refuse any part of the examination and/or treatment

No procedure can be performed without your informed consent.

IMPORTANT TO KNOW

You can withdraw your consent at any time.

What is informed consent?

Informed consent means that you know:

- **what is being done**
- **why it is being done**
- **the benefits and possible risks**
- **what alternatives exist**
- **what happens if you refuse the exam or treatment**

Consent is not valid if it is given:

- **under pressure from the gynecologist**
- **under coercion or fear**
- **without a clear explanation**
- **by signing a form you do not understand**

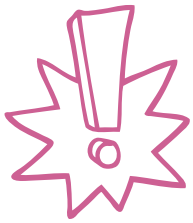
What if you feel scared, ashamed or unsafe?

Such feelings are **common** and **understandable**.

Discomfort can be a normal experience but:

A feeling of unsafety is a signal that something is not right.

If this happens, you can:



- stop the exam
- request a different gynecologist
- ask for the presence of a companion
- report inappropriate behavior

If the exam was distressing for you, it is important to talk about it with someone you trust!

Empathy is not a luxury. It is an important and essential part of healthcare.

We asked a psychologist

How can we teach young people that they have the right to say “no,” even in a medical setting?

“We may need to repeat it endlessly until it sticks for all of us: **Everyone has the right to be assertive about their health**. It feels like we have to remind ourselves of this a thousand times.

Every person deserves a dignified exam.
A doctor **does not have** more rights than we do. The fact that we are not doctors does not mean we cannot recognize when something is inappropriate during an examination (I am referring to the tone of communication and pain).”

What would be your message to young people who avoid examinations because they have had a painful previous experience?

“**A yearly exam can save lives**.
Find a way, with the support of people around you, to gather the courage to go for another check-up. There is a map of gynecologists who are considered good.
Try to find such a doctor in the nearest town or city to you.”

dr Mina Hagen



After the gynecological exam

What can I expect after the exam?

After the exam **you have the right to know**:

- when and how you will receive your results (in person or by email, depending on the healthcare facility)
- whether any further treatment is needed
- what the next recommendations or follow-up appointments are

If further treatment is needed, the gynecologist is required to:

- explain the reason for it
- provide guidance and advice
- refer you to the next steps

At your first examination, you also have the right to:

- receive advice on intimate health and hygiene
- receive information that is appropriate for your age and experience level

If the exam feels difficult, frightening or stressful:

- bring a trusted person with you (a parent, friend, partner, etc.)
- ask for a recommendation for a gynecologist from someone who has had a positive experience

What are your rights after the exam?

You have the right to:

- access your medical records
- receive an explanation of your diagnosis and treatment
- refuse treatment
- ask about alternative treatment options
- seek a second medical opinion
- privacy and confidentiality of your personal health information

How can a negative experience affect your future health?

Negative experiences can lead to:

- avoiding future examinations
- a disrupted relationship with your body
- feelings of shame and self-doubt about your experience of pain
- difficulties in sexual relationships

Experiences are especially harmful when:

- pain is dismissed or minimized
- the person is judged by a healthcare professional
- a person's identity or sexual orientation is not respected (discrimination)

How does Serbia recognize and address discrimination in healthcare?

Serbia has laws that formally prohibit discrimination on the grounds of gender identity, sexual orientation, disability, and other protected characteristics.

In practice:

- institutions often do not know how to respond appropriately or may refuse to act
- cases are rarely reported, which means there is limited legal practice and case law in this area

If discrimination occurs, the case can be reported to:

- the Commissioner for the Protection of Equality
- the Commissioner for Information of Public Importance and Personal Data Protection
- the Protector of Citizens (Ombudsman)
- the health inspection authorities
-

Before initiating court proceedings, the following order is generally recommended: legal advisor → Commissioner for the Protection of Equality → Ombudsman → health inspection.

If the issue is not resolved, the person may file a **lawsuit** or **submit a criminal complaint**.

A higher number of reported cases can contribute to changes in institutional practices, public awareness, and institutional decision-making.

We asked a lawyer

What are the basic rights of young people during a gynecological exami?
Can young people refuse a part of the exam and how can they do so without consequences?

Young people have the right **to be heard**, to ask questions, and to receive answers in language and terms they can understand.

They have the right to **respect**, **communication**, and **dignified treatment**, not a “factory-like” approach in which they feel invisible or unimportant.

They have the right to **privacy** and, from the age of 15, to go to a doctor **on their own**, without mandatory company.

They have the right **to choose who is present** during the exam (a trusted person), if they wish, and no one may deny them this.

They have the right to be addressed by their **name** during the exam, rather than through stereotypical or impersonal forms of address.



Marina Mijatović (lawyer and MP of the Green–Left Front)

What is menstruation?

Menstruation is monthly bleeding that occurs when the lining of the uterus sheds because pregnancy has not occurred.

Menstruation is a natural part of the cycle that repeats every 21 to 35 days.

Menstrual cycle is the period from the first day of one menstruation to the first day of the next menstruation, i.e. menstrual bleeding.

Cycle length is the number of days between the start of two consecutive menstrual periods.

Duration of menstrual bleeding is the number of days from the beginning to the end of bleeding.

Menstruation is a natural bodily process that varies from person to person.

Example: Menstruation started on March 5 – the next menstruation started on April 2.
This means: Menstrual cycle = the period from March 5 to April 1.
Cycle length = 28 days

What is the menstrual cycle?

It is a common misconception that the menstrual cycle begins and ends with menstrual bleeding.

The menstrual cycle is not just the period from the beginning to the end of menstrual bleeding, but encompasses a whole series of changes that take place in the body.

Menstruation is just one of the four phases of the menstrual cycle. Each of these phases has its own changes and role in the menstrual cycle.

The menstrual cycle is divided into four phases:

Menstruation (days 1–7): The first day of the menstrual cycle is the first day of menstrual bleeding. This is the period when the lining of the uterus, which built up during the previous cycle, is shed from the body because pregnancy has not occurred.

Follicular phase (days 1–14): This phase overlaps with menstruation because it includes all the days before ovulation. The ovaries develop follicles (fluid-filled sacs in which egg cells mature), and estrogen levels gradually rise.

Ovulation (days 14–15): When the level of the hormone (LH) rises sharply, a mature egg cell is released from the ovary. This is ovulation, which lasts only 12 to 24 hours, during which the egg cell travels towards the uterus and can be fertilized.

Luteal phase (days 15–28): This is the period after ovulation, up until the first day of the next menstrual bleeding. During this phase, progesterone levels rise, the body prepares for a possible pregnancy, and if fertilization does not occur, hormone levels drop and the menstrual cycle begins again. PMS (premenstrual syndrome) symptoms occur during this phase.

This is just one example of what a menstrual cycle can look like. Every body is different, which means that menstrual cycles can also vary.

IMPORTANT TO KNOW

PMS is a combination of physical and emotional changes that occur in the body. They may appear a few days before menstruation.

Some menstrual hygiene products you can use during your period:

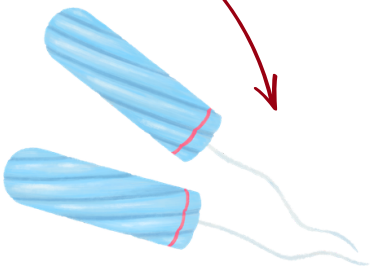
Sanitary pads



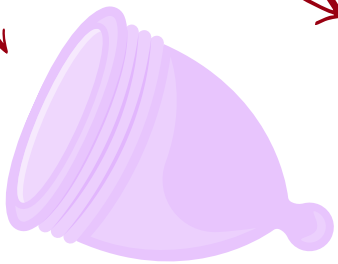
They are attached to your underwear and should be changed every 3 to 4 hours, or more often depending on the heaviness of the bleeding.

Tampons

They are inserted through the vaginal opening and should be changed every 4 to 8 hours, depending on the heaviness of the bleeding.



Menstrual cup or menstrual disc



It is folded and inserted through the vaginal opening, where it collects menstrual blood. Depending on the product and the heaviness of the flow, it can typically be worn for up to 12 hours before being emptied, cleaned, or replaced according to the manufacturer's instructions.

Menstrual underwear

They can be worn instead of or together with other menstrual hygiene products and should be changed when they become damp or fully saturated.



Tips for managing menstrual discomfort

Common symptoms in the days before and during menstruation include cramps, bloating, headaches, mood swings, breast tenderness, lower back pain, fatigue, and sleep problems. Here are some ways to relieve them:

Lifestyle: Regular physical activity, a balanced diet, enough sleep, and activities that reduce stress can help make PMS symptoms easier to manage.

Supplements (only on a doctor's advice):

- **Magnesium** may help with headaches and cramps.
- **Calcium** may help reduce fatigue and mood swings.
- **Vitamin B6** may help with irritability, bloating, and concentration.
- **Brufen/Caffetin Menstrual** may help reduce pain in the abdomen, breasts, and back.

A warm bath, a hot water bottle, and herbal teas can also help relieve painful cramps. If symptoms do not improve with these measures or significantly interfere with daily life, this is a sign that you should seek help from a gynecologist!

Symptoms that should not be ignored

Severe pain, very irregular menstruation, or symptoms that interfere with daily life are just some of the signs that it is **necessary** to see a doctor.

It is important to know the difference between common discomforts and symptoms that may indicate more serious gynecological conditions,
such as:

- **Polycystic Ovary Syndrome (PCOS):** Irregular or missed periods, acne, excessive hair growth, and sudden changes in body weight.

- **Endometriosis:** Very severe menstrual pain that does not improve even with the help of common painkillers, pain during sexual intercourse, as well as pain when urinating or having a bowel movement.

- **Luteal Phase Defect (LPD):** A luteal phase that is too short, which may affect the ability to conceive.

- **PMDD (Premenstrual Dysphoric Disorder):** A more intense form of PMS that significantly affects mood and daily life.

- **Irregular or heavy periods:** Very prolonged or heavy bleeding, as well as bleeding between periods.

Menstrual misconceptions, myths and taboos

Young people often receive **misinformation** from those around them when it comes to menstruation. Some of the most common myths are:

- ✕ **"You shouldn't shower or take a bath during menstruation."** This statement is false because hygiene during menstruation is extremely important. Warm water can even help relieve cramps.
- ✕ **"You can't get pregnant during menstruation."** Although the likelihood is lower, pregnancy is still possible because ovulation can overlap with menstruation.
- ✕ **"Menstrual pain is something you simply have to endure."** This is false. Mild discomfort and pain are common, but severe, debilitating pain should be reported to your doctor.
- ✕ **"A normal cycle must always start on the same date."** The menstrual cycle does not always last the same amount of time, and the start date of menstruation may vary each month. However, significant changes and irregular cycles should be reported to a doctor.
- ✕ **"PMS is 'made up' or exaggerated."** This is false. PMS has a clear hormonal basis and affects most people who menstruate to varying degrees.

Menstruation is a natural and completely normal occurrence.

The shame and discomfort surrounding it are learned, but that doesn't mean we actually need to feel them. When we experience these feelings, it becomes harder to talk about pain and other symptoms, ask for help when we need it, or speak up when we don't have access to menstrual products.

This can also lead to the use of improvised or inadequate alternatives, which carry serious health risks.

We call this problem period poverty.

Period poverty is not just a lack of access to menstrual products, but also to adequate hygiene, education, or sanitary facilities due to financial or other barriers.

When we talk about menstruation more openly, we create an environment where it is easier to ask for help, support, advice, or a menstrual product when we need one.

Every one of us is here today thanks to someone who menstruates, yet menstruation is still a taboo subject among young people.
It is up to all of us to finally change that.

Glossary of Terms

What is a speculum?

A speculum is a medical instrument (made of metal or plastic) used to gently open and separate the walls of a body cavity, such as the vagina, allowing the gynecologist to examine the inside of the vagina and the cervix, as well as collect swab samples.
Although a speculum examination may feel somewhat uncomfortable, **it should not be painful.**

There are different sizes of speculums available, and you have the right to ask for a different size if the one being used is not comfortable for you.



What is a Pap smear/test?

A Pap (Papanicolaou) test is a simple procedure in which a small sample of cells is collected from the cervix to detect early changes that may require monitoring or treatment.



What are HPV infection and the HPV vaccine?

HPV (Human Papillomavirus) is a group of viruses that are transmitted through sexual contact. They can infect the skin and mucous membranes and may lead to the development of abnormal cells and cancers of the cervix, vulva, vagina, penis, anus, mouth, or throat. Importantly, many HPV infections and related diseases can be prevented through vaccination. If you would like to learn more about this important topic, you can visit hpvinfo.rs and/or talk to a gynecologist about what is best for you.

What is contraception?

Contraception is a set of methods and devices used to prevent conception, or an unintended pregnancy. It works by preventing ovulation, fertilization, or the implantation of a fertilized egg in the uterus.

There are different types of contraception:

- **Barrier methods** (condoms), the only method that also protects against sexually transmitted infections.
- **Hormonal methods** (contraceptive pills, patches, vaginal ring, injections), which act on the hormonal cycle and prevent ovulation.
- **Intrauterine devices** (IUDs), hormonal and non-hormonal, which prevent fertilization and/or implantation.
- **Emergency contraception**, used after unprotected intercourse within a limited time frame.

The choice of method depends on a person's health status, lifestyle, age, and personal needs, and it is important that it is made in consultation with a gynecologist.

Conclusion

This guide is an educational resource created with the aim of **empowering individuals who are going for a gynecological examination for the first time and improving gynecological healthcare practice.**

The content of the guide was prepared by the project authors, with advisory support from
Prof. Dr. Aleksandra Kapamadžija – gynecologist, Dr. Mina Hagen – psychologist,
and Marina Mijatović – lawyer and Member of Parliament of the Green–Left Front.

The information in this guide is based on current professional knowledge, clinical experience, and the experiences of users of the healthcare system.

This handbook is not a substitute for individual medical advice, diagnosis, or treatment, but serves as an informative and supportive guide. In case of health concerns, consultation with qualified healthcare professionals is recommended.

Recommend gynecologists map (for Serbia):
https://biopolis.rs/portfolio_page/mapa-preporuka/



Belgrade, 2026.